AN EXPLAINER IN TWO HALVES

The record arrives as a letter.

Five slides on what breaks in UK care data. Five on the build that closes it.

FHIR R4 | SNOMED CT | NHS NUMBER | 10 AGENTS

INDEPENDENT PROJECT · NO PATIENT DATA · NOT AFFILIATED WITH THE NHS

Four records. Four owners.

GP practice

Independent contractor. EMIS Web or SystmOne holds the lifelong record.

Hospital trust

Its own electronic patient record — separate system, separate login.

Pharmacy

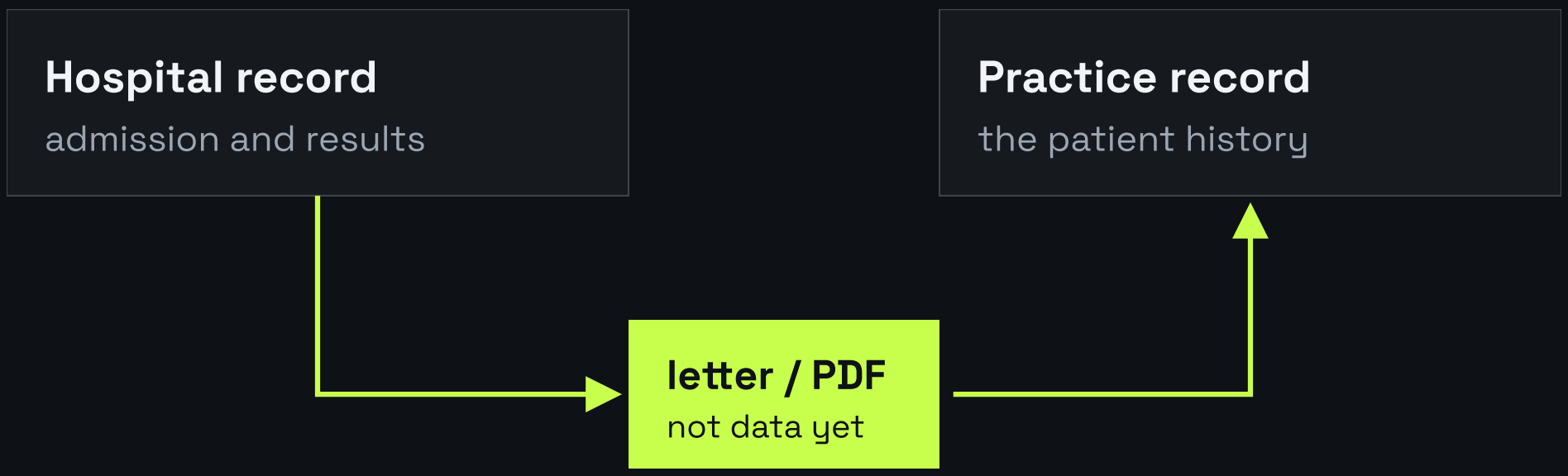
Its own dispensing record, usually unconnected to the practice.

National

NHS number, Spine, GP Connect — plumbing, not the record.

No single patient record exists. Several do.

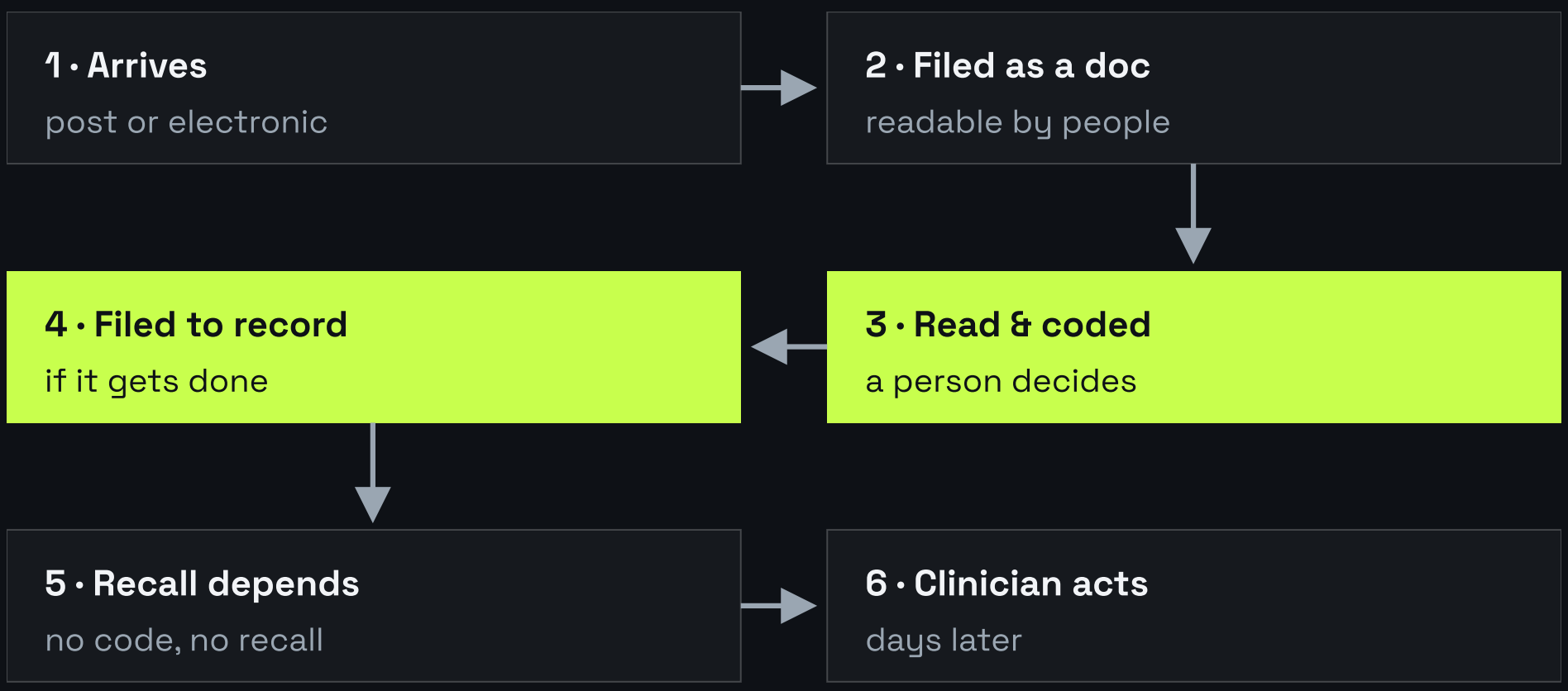
So what joins them?



A PERSON READS IT AND CODES IT

UNTIL THEN THE RECORD DOES NOT KNOW

Six hands.
Two are manual.



STEPS 3 AND 4 CARRY A HUMAN, IN SEQUENCE, EVERY TIME

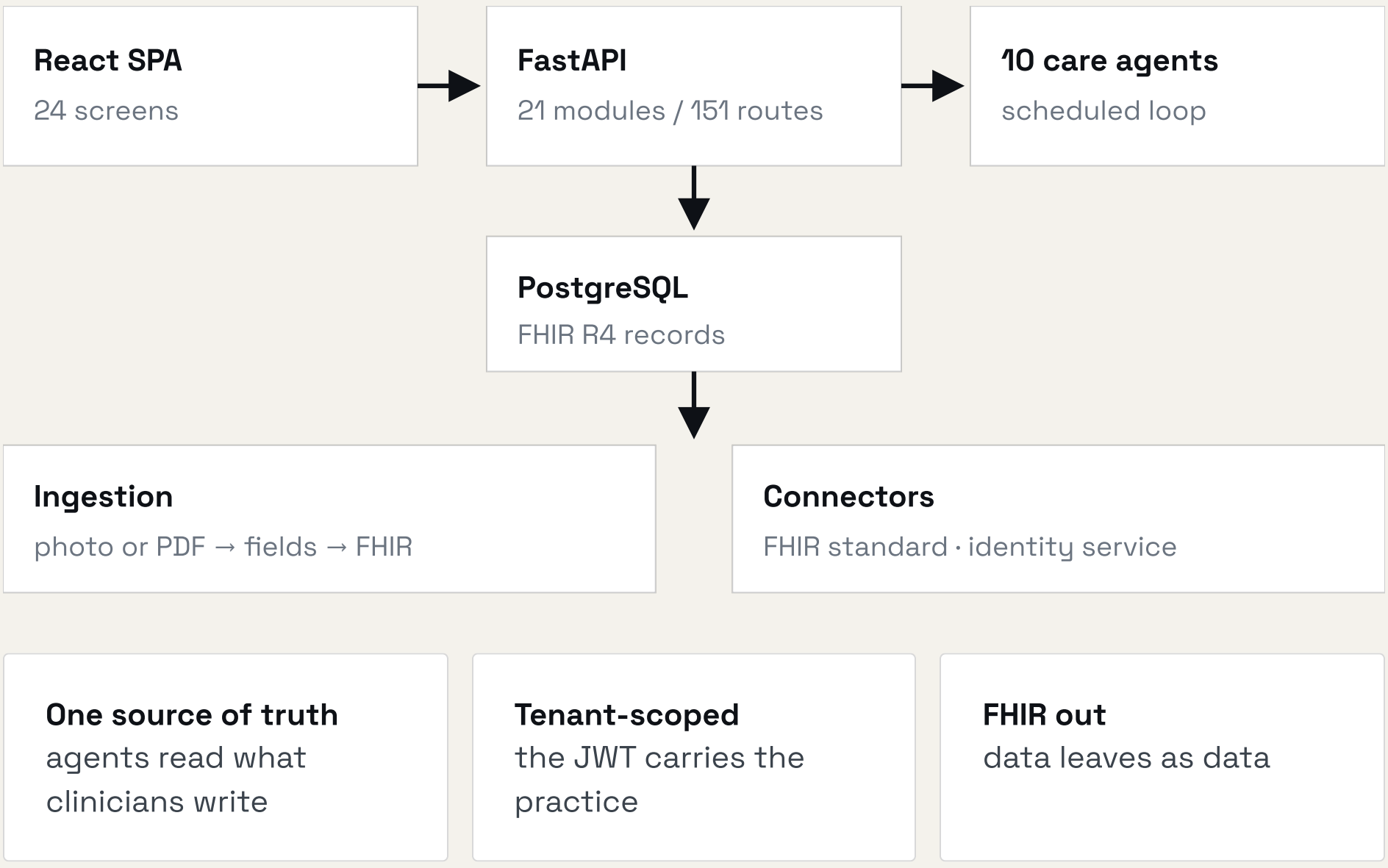
What that costs

- **Coding lag** — the fact is on paper, not in the record being searched.
- **Recall misses** — a recall queries codes; free text is invisible.
- **Duplication** — the same datum typed again into a second system.
- **Delay** — a result in an inbox behaves like one that never arrived.

None of it is a person failing. It is a system with a manual seam.

THE BUILD

Structure of the running code

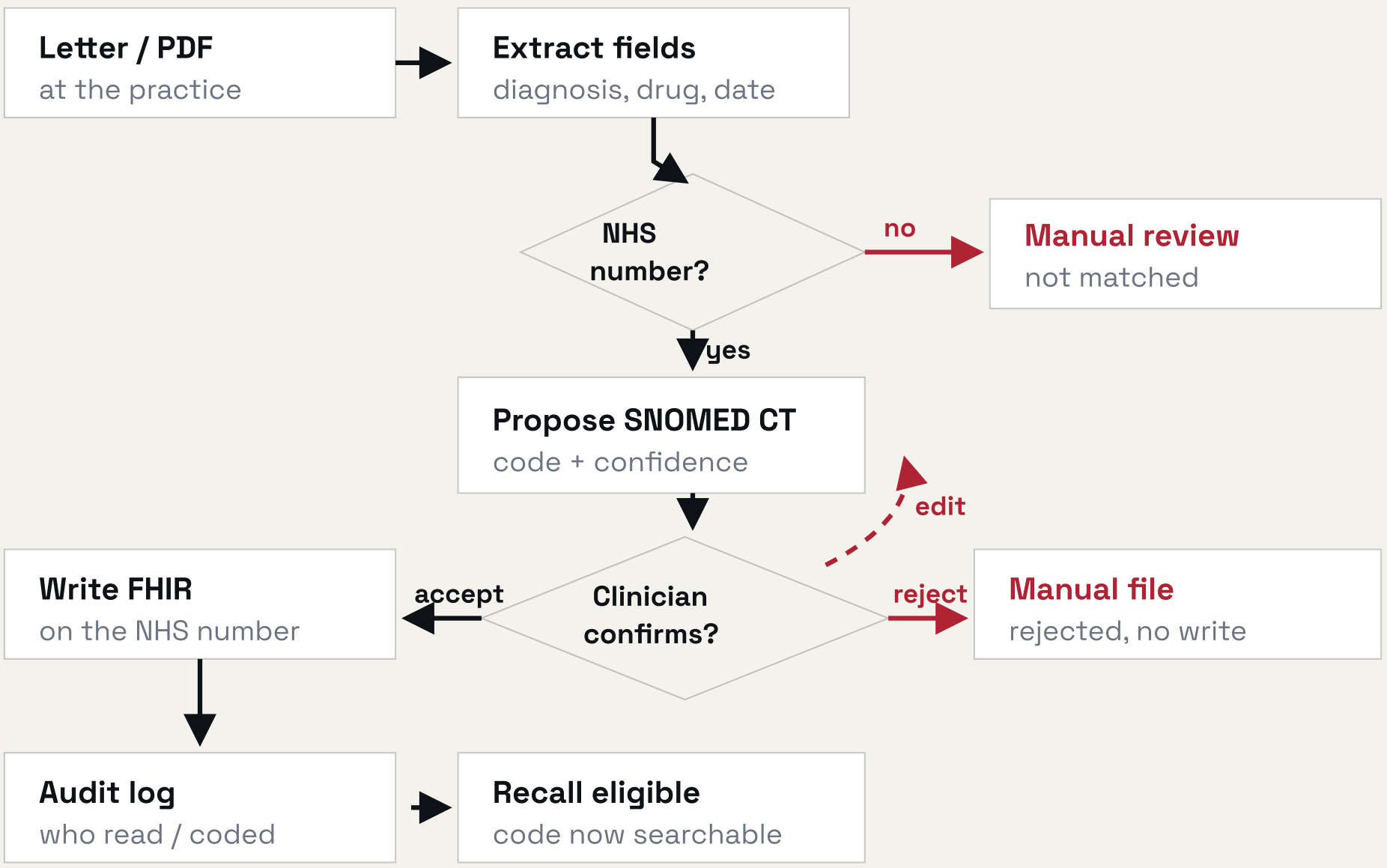


Written for India. Mapped for the UK.

IN THE CODE	UK EQUIVALENT
ABHA / UHID	NHS number
ICD-10	SNOMED CT (SCCI0034)
FHIR R4 base	FHIR UK Core
DPDP 2025	UK GDPR + DPIA
WhatsApp bridge	NHS App messaging

The engineering travels — tenant isolation, audit trail, human confirmation. The terminology and certifications are UK work.

One letter, three outcomes



Only **accept** writes to the record.

WHAT EXISTS TODAY

151
REST endpoints
across 21 modules

24
clinical screens
React + TypeScript

10
care agents
on a schedule

33/45
build tasks
complete

THE AGENTS

patient intake · appointments · medication adherence · follow-up · labs · pharmacy · insurance · risk prediction · family care · voice care

What it will not do — or claim

No diagnosis

No autonomous clinical decision.

No shared pool

No cross-practice patient data.

No silent writes

Every filed fact is attributable.

No certificates

DTAC, DSPT, DCB0129 not held.

SEE THE LOOP — SIX SCREENS, SAMPLE DATA

19p1362.github.io/portfolio/healthbridge

GONELA ABHILASH · CPC · REGISTERED PHARMACIST · MSC PUBLIC HEALTH, ARU